BASELINE → TREATMENT → FOLLOW-UP

Your tests and treatment checks

Use the individual prescription and appointment calendar. No universal tumour-marker panel or scan interval is assumed.

Check	Before starting	Early treatment	Ongoing checks	Dose hold / restart	After treatment
Blood and organ safety	Full blood count, kidney and liver tests and thyroid function; electrocardiogram (ECG) if at risk of heart-rhythm changes; international normalised ratio (blood-clotting test) (INR) if taking warfarin. Review cardiovascular and seizure history, falls, bone health and all medicines.	Repeat blood count, kidney and liver tests on the schedule agreed with your team. Thyroid function and ECG as clinically indicated. If warfarin is unavoidable, INR weekly until the dose is stable. The National Cancer Control Programme (NCCP) regimen does not set one fixed interval for every routine blood test.	Extra tests may be needed for symptoms, dehydration or abnormal results.	Review results, recovery and previous toxicity before any restart or next dose.	Continue your prescribed androgen deprivation therapy (ADT), unless you have had surgery to remove both testes. The team reviews symptoms, blood tests, PSA and scans according to your own plan; there is no universal scan interval.
Appointments and medicines	Agree the treatment aim, dose and complete medicines list.	Take at the same time each day, with or without food. Swallow tablets whole with water. If a dose is missed, take it as soon as possible that day and return to the usual schedule the next day; never take extra tablets to make up a dose. If you cannot swallow the tablet, a pharmacist must teach the correct preparation for your 60 mg or 240 mg formulation or feeding tube.	Keep a current dose and symptom record.	Report rash promptly. Blisters, peeling skin, fever with rash, or mouth/eye sores need immediate medical advice. New or worsening cough or breathlessness requires urgent assessment for lung inflammation. Seizures require emergency help. Discuss driving and medicine interactions.	Keep follow-up and recovery checks even after the last dose.
Cancer-specific and specialist tests	PSA and testosterone monitor cancer activity and ADT suppression. PSA doubling time helps select high-risk nonmetastatic castration-resistant disease. Thyroid function is a safety test because apalutamide can cause an underactive thyroid.	Confirm the booked test dates with your team.	Safety tests and response tests have different purposes.	Continue your prescribed androgen deprivation therapy (ADT), unless you have had surgery to remove both testes. The team reviews symptoms, blood tests, PSA and scans according to your own plan; there is no universal scan interval.	Agree the named follow-up team and remaining checks.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00574 v5: official regimen

Tests in your plan

Baseline bloods and tests

WHEN

Full blood count, kidney and liver tests and thyroid function; ECG if at risk of heart-rhythm changes; INR if taking warfarin. Review cardiovascular and seizure history, falls, bone health and all medicines.

Checks organ safety and suitability before treatment.

Treatment safety monitoring

WHEN

Repeat blood count, kidney and liver tests on the schedule agreed with your team. Thyroid function and ECG as clinically indicated. If warfarin is unavoidable, INR weekly until the dose is stable. The NCCP regimen does not set one fixed interval for every routine blood test.

Finds changes that may need supportive care, a dose hold or adjustment.

Specialist and cancer tests

If part of your plan

WHEN

PSA and testosterone monitor cancer activity and ADT suppression. PSA doubling time helps select high-risk nonmetastatic castration-resistant disease. Thyroid function is a safety test because apalutamide can cause an underactive thyroid.

Distinguishes treatment selection, hormone monitoring and cancer response.

THE WHOLE TREATMENT JOURNEY

Apalutamide: prostate cancer with ongoing ADT: treatment and checks

High-risk nonmetastatic castration-resistant prostate cancer, or metastatic hormone-sensitive prostate cancer, together with ADT. Your team confirms the indication and its separate eligibility criteria.

Apalutamide 240 mg once daily, continuously while helping and tolerated, with ongoing ADT unless surgically castrated. Lower prescribed doses are 180 mg or 120 mg daily when indicated.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Early treatment	Ongoing checks	Dose hold / restart	After treatment
Blood and organ safety NCCP	Full blood count, kidney and liver tests and thyroid function; ECG if at risk of heart-rhythm changes; INR if taking warfarin. Review cardiovascular and seizure history, falls, bone health and all medicines.	Repeat blood count, kidney and liver tests on the schedule agreed with your team. Thyroid function and ECG as clinically indicated. If warfarin is unavoidable, INR weekly until the dose is stable. The NCCP regimen does not set one fixed interval for every routine blood test.	Extra tests may be needed for symptoms, dehydration or abnormal results.	Review results, recovery and previous toxicity before any restart or next dose.	Continue your prescribed androgen deprivation therapy (ADT), unless you have had surgery to remove both testes. The team reviews symptoms, blood tests, PSA and scans according to your own plan; there is no universal scan interval.
Appointments and medicines NCCP	Agree the treatment aim, dose and complete medicines list.	Take at the same time each day, with or without food. Swallow tablets whole with water. If a dose is missed, take it as soon as possible that day and return to the usual schedule the next day; never take extra tablets to make up a dose. If you cannot swallow the tablet, a pharmacist must teach the correct preparation for your 60 mg or 240 mg formulation or feeding tube.	Keep a current dose and symptom record.	Report rash promptly. Blisters, peeling skin, fever with rash, or mouth/eye sores need immediate medical advice. New or worsening cough or breathlessness requires urgent assessment for lung inflammation. Seizures require emergency help. Discuss driving and medicine interactions.	Keep follow-up and recovery checks even after the last dose.

Care / check	Before starting	Early treatment	Ongoing checks	Dose hold / restart	After treatment
Cancer-specific and specialist tests NCCP	PSA and testosterone monitor cancer activity and ADT suppression. PSA doubling time helps select high-risk nonmetastatic castration-resistant disease. Thyroid function is a safety test because apalutamide can cause an underactive thyroid.	Confirm the booked test dates with your team.	Safety tests and response tests have different purposes.	Continue your prescribed androgen deprivation therapy (ADT), unless you have had surgery to remove both testes. The team reviews symptoms, blood tests, PSA and scans according to your own plan; there is no universal scan interval.	Agree the named follow-up team and remaining checks.

What each check is for

1

Treatment and review

Once daily plus ongoing ADT

This treatment aims to slow cancer growth and help control symptoms. Your team explains the expected benefit, uncertainty and alternatives for your situation.

Ask / act: Repeat blood count, kidney and liver tests on the schedule agreed with your team. Thyroid function and ECG as clinically indicated. If warfarin is unavoidable, INR weekly until the dose is stable. The NCCP regimen does not set one fixed interval for every routine blood test.

Sources: NCCP

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-14 · Source reconciled draft; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.51